

“MANAGEMENT OF ARDHAVABHEDAKA W.S.R.TO MIGRAINE - AN OBSERVATIONAL STUDY”



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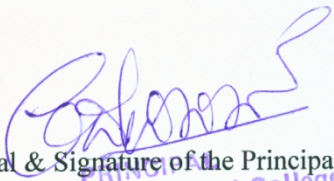
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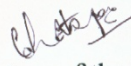
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Dedicated to.....

My beloved Parents, Teachers and Friends....

With immense love, respect and gratitude

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Dr.CHITHRALATHA

LIST OF ABBREVIATIONS

A.B.M.L.S.	-	Akshi, Bhru, Manya, Lalata, Shanka
A.Hr.Su	-	Astanga Hridaya Sutrasthana
A.Hr.Ut	-	Astanga Hridaya Uttaratanttra
A.S	-	Astanga Sangraha
A.T	-	After Treatment
B.B.B	-	Blood Brain Barrier
B.N.R.	-	Brihat Nighantu Ratnakara
Bha.Pra/BP	-	Bhava Prakasha
Ba.Pr.Ma	-	Bhava Prakasha Madhyama khanda
Ba.Ra	-	Basavarajeeyam
Bh.Ra	-	Bhaishajya Ratnavali
B.S	-	Bhela Samhita
CS	-	Charaka Samhita
C.D	-	Chakra Datta
C.M.D.T	-	Current Medical Diagnosis & Treatment
Cha.Si	-	Charaka Siddhithana
Cha.Su	-	Charaka Sutrasthana
Cha.Chi	-	Charaka Chikitsa
D.G.V	-	Dravya Guna Vijnana
DOP	-	Duration of pain
FOA	-	Frequency of Attack
G.N	-	Gada Nigraha
H.S	-	Hareeta Samhita
HS	-	Highly Significant
Ma.Ni /MN	-	Madhava Nidana
Ni	-	Nidana sthana
R.Y.S	-	Rasa Yoga Sagara
SS	-	Sushruta Samhita
S.E.D	-	Sanskrit English Dictionary – M.M.Williams

S.K.D	-	Shabda Kalpa Druma
S.M.D	-	Stedman's Medical Dictionary
Su.Sa	-	Sushruta Samhita
Su.Sa.Ut	-	Sushruta Samhita Uttaratantra
S.Y	-	Sahasra Yoga
SS	-	Statistically Significant
Su	-	Sutra sthana
T.B.Ph	-	Text Book of Physiology
T.B.Pha	-	Text Book of Pharmacology
T.B.Ps	-	Text Book of psychiatry
T.B.M	-	Text Book of Medicine
Ut	-	Uttara tantra
V.S	-	Vangasena Samhita
Vch	-	Vachaspatyam
Y.R	-	Yoga Ratnakara

ABSTRACT

BACKGROUND AND OBJECTIVES

Migraine is a common clinical problem characterized by episodic attacks of head pain and associated symptoms such as nausea, sensitivity to light, sound, or head movement.

It is generally thought of as a headache problem, but it has become apparent in recent years that many patients suffer symptoms from Migraine, which do not have severe headaches as a dominant symptom.

The number of people suffering with atypical forms of Migraine is unknown. Females are 3 times more likely to have Migraine than males. Although any person can have Migraine at any age, Migraine is most common between ages 30 and 50.

It is a common condition with a prevalence of 17.6% in females and 5.7% in males; however, this condition is under-treated and under-diagnosed worldwide.

Treatment of Migraine has not only medical but also serious economic and social implications. Thus, primary-care physicians should be well versed in the diagnosis and treatment of Migraine.

The objectives of the study were to assess the combined effect of Kumkuma Ghrita Nasya, Tab. Kamadugha and Tab.Siledin in the management of Ardhavabhedaka

Method:

Total 50 patients, having symptoms of Ardhavabhedaka were included.

50 patients were assigned in a single Group and all patients were administered with Kumkuma Ghrita Nasya for 7 days along with Tab. Kamadugha and Tab.Siledin for 30days.

Data was collected before commencement of treatment, after Nasya karma, on 20th day and on 30th day of study period.

Results were statistically analyzed before and after the treatment by using Chi-square test/configurancy table analysis, Descriptive statistics, Cross tabs procedure, paired sample 't' test. Repeated measure ANOVA/ Repeated measures using SPSS for windows software.

Results:

Combined effect of drug showed highly significant and statistically significant results. Significant results were obtained on severity and duration of headache, frequency of attack with significant relief from associated symptoms by the end treatment period. There was good improvement in stress scores at the end of treatment. Overall assessment showed complete, marked and moderate improvement in most of the patients.

Interpretation and Conclusion:

The treatment modality adopted was highly effective on Ardhavabhedaka; most of the patient's experienced Maximum benefit at the end of treatment schedule.

Key words:

- Ardhavabhedaka.
- Migraine
- Kumkuma Ghrita
- Nasya
- Tab. Kamadugha
- Tab.Siledin

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INTRODUCTION

The great economist Adam Smith has told that the needs of human being are infinite but the availability is less to fulfill. Our life style has been drastically changed and our growing needs have no end. Speed and accuracy are the prime demands of modern era. To cope with this situation everybody have to face hectic, competitive and stressful life.

People cannot pay attention to their physical and mental health. Irregular food habits, suppression of natural urges, lack of proper sleep and less time for relaxation are being part of our life, which enhances incidences of many diseases mainly having psychosomatic origin at a drastic speed.

Headache is 9th most common complaints of the people seeking professional help. Only few of us are spared the experience of headache. 90% of the individual have at least one headache per year, 10 to 20% of the population goes to physician with headache as their primary complaint. It is also a major cause of absenteeism from work and avoidance of social and personal activities.

Severe headache attacks regardless of cause are more likely to be described as throbbing and associated with vomiting and scalp tenderness. It is ranked by W.H.O as number 19 among all diseases worldwide causing disability.

Vascular headaches, a group that includes Migraine, are thought to involve abnormal function of the brain's blood vessels or vascular system. Migraine is a common disabling paroxysmal headache usually characterized by severe pain on one or both sides of the head, an upset stomach, and, for some people, disturbed vision. Though vascular changes are evident during a Migraine, the cause of the headache is neurologic.

35% of headache sufferers are from Migraine headache as it is common problem in India and Abroad in these days. Approximately 15% of women and 6% of men of the population are afflicted by Migraine. Women get more Migraine than men because of hormonal fluctuations, and the incidence appears to be increasing recently. The age group between 30 and 45 has the peak incidence and about 4% of children have Migraine.

Shiras is considered as uttamanga as it is the seat of prana and all indriyas. It is also considered as one among trimarmas. The Kayachikitsa deals with the diseases of whole body including the uttamanga like jwara, raktapitta, apasmara, unmada etc.

The two angas i.e., kaya and urdhwanga, have some of the common vyadhis and are inter-related. One such disease is Ardhavabhedaka. It is one among the 11 types of Shirorogas considered by Acharyas.¹ Even though it troubles the uttamanga; it requires medication for the whole body to get rid of the trouble.

A glimpse on the history of Ardhavabhedaka is not established in Vedakala, but the word 'shirashakti' was used to denote headache and Nasya was the treatment mentioned in Atharvaveda. Aside from this, the Ardhavabhedaka is mentioned in all major Samhita of Ayurveda.

The causes of Ardhavabhedaka are excessive intake of rooksha padarthas, Adhyashana, Purva vata sevana, Atimaithuna (excessive coitus), Vegadharana (suppressing of natural urges), Athishrama (excessive work) in which pain is appreciable in one half of the shiras, shanka, bru, manya, akshi, and karna pradesha.²

The attacks of Ardhavabhedaka will be once in three days, once in fifteen days and once in a month as per classics.³ Thus, it can be correlated with Migraine based on the similarity in etiology, symptoms and treatment principles.

As per Acharya Charaka, it is said that Ardhavabhedaka if left untreated it leads to diseases of deafness and blindness.⁴ Migraine does not shorten the life, but in severe cases a state of chronic exhaustion may occur. Very rarely persistent cerebral symptoms remain with some irreversible vascular changes.

It is medical and health problem of first magnitude, because of the frequency, the vast span of clinical types, the known and unknown causes and mechanisms, number of work hours lost by those suffering with headache.

Treatment of this condition available to present day include, use of analgesics, and vasodilators have insignificant role in achieving the success in addition to their adverse effects. Owing to the above mentioned problems of management, it is imperative to explore newer, efficacious drugs or procedures to tackle disease

Migraine.

Ayurveda classics have mentioned various therapeutic measures in the management of Ardhavabhedaka. Nasya karma is considered as an important modality of treatment for all types of Shirorogas . i.e. '*Nastah karmacha kurveeta shirorogeshu shastravid*'

Therefore, this study has been intended to evaluate the combined effect of Kumkuma Ghrita Nasya, Kamadugha Rasa and Tab.Siledin in the management of Ardhavabhedaka.

The present work comprises of two parts. First part deals with Review of Literature on Ardhavabhedaka and Migraine, Drug Review, Review on Previous works. Second part consists of Methodology, Observation, Results, Discussion, Conclusion, and Summary of clinical research work done and documented in respective chapters.

OBJECTIVES

Migraine is a benign and recurrent syndrome of headache, nausea, vomiting and other symptoms of neurological dysfunctions in varying admixtures.

The symptoms complex is very much similar to that of Ardhavabhedaka. Hence the particular disease troubling most of the people is selected for the study.

Migraine is a chronic disease, does not shorten the life in early condition. But later may cause cerebral as well as vascular complications.

The severity of shoola is so strong that it breaks the interest of one's life and diverts his attention from his routine normal activities and makes one restless. Shiras is considered as an uttamanga and it is one among trimarmas.⁵ Thus, the disease affecting the shiromarma, it requires special interventions.

Though different medicines are explained in the science, there is a lacuna for an appropriate treatment in Ardhavabhedaka. Hence the present study aims to fulfill these needs. In this regard while exploring ayurvedic classics, we come across few simple techniques and medicaments in the management of Ardhavabhedaka.

Title of the present study:

“Management of Ardhavabhedaka with special reference to Migraine”

Objectives of the study:

- ❖ To evaluate the combined effect of Ghritayukta kesara (Kumkuma Ghrita) Nasya, Kamadugha Rasa and Tab.Siledin in the management of Ardhavabhedaka.

REVIEW O LITERATURE

VYUTPATTI, NIRUKTI AND PARIBHASHA

Vyutpatti

The word Ardhavabhedaka is derived from three words (i) ardha (ii) ava and (iii) bhedaka.

- I. Word ‘Ardha’ formed from ‘Rudh dhatu’ and ‘dhan pratyaya’. In masculine gender it means samanamsha / vibhaga/ samabhaga and in feminine gender it carries the meaning as ‘Ekadesha’. Thus, “Ardha” – means half/ half part/ halved/ forming a half.⁶
- II. ‘Ava’ is used as visarga pada as it is derived from ‘ava’ dhatu and ‘ach’ pratyaya.⁷
- III. ‘Bhedaka’ word is formed from ‘bhid dhatu’ and ‘yevul pratyaya’ which means vidaraka or bhedaka/kutarena vidaranavat vedana has got all three genders.⁸

It is a verb suggesting an action of beating/piercing/hitting.⁹

Thus, the compound word ‘Ardhavabhedaka’ gives the meaning of breaking, cutting or dividing into halves.¹⁰

Hence the disease Ardhavabhedaka mainly indicates shira shoola which is of breaking / splitting type/piercing type.

Nirukti and Paribhasha

As per Sushruta,

Yasyottamanga ardhamateeva jantoh sambheda toda bhramashoolajushtam|

Pakshaat dashahaat athava apyakasmaat tasyardhabhedam tritayaat vyavasyet||¹¹

If one half of the head develops severe tearing, piercing and pricking pain and giddiness suddenly after a fortnight or ten days then this should be diagnosed as

Ardhavabhedaka. It is caused by all the three doshas.

According to Vagbhata.... *Ardhe tu moordhnah so Ardhavabhedakah*¹²

The definition of Ardhavabhedaka which is available in the commentaries on Bhrihatrayee's as.... *ardhe tu shirasah Ardhavabhedaka uchyate*

Hence, Ardhavabhedaka is the disease producing piercing type of pain in the ardha Shiras or ardha moordhna pradesha.

Migraine:

The term Migraine stems from Galen's usage of 'Hemicrania' to describe a periodic disorder. Hemicrania was later corrupted into low Latin as 'hemigrania' and 'migrania'. Eventually the French cognate, Migraine gained acceptance in the 18th century and has prevailed ever since.¹³

A symptom complex occurs periodically and characterized by pain in the head (usually unilateral), associated with vertigo, nausea and vomiting, photophobia, Phonophobia and scintillating appearance of light.

The word Migraine is derived from the Greek word mi-gran (through 0.7r fr. A. hemikrania pain on one side of the head, fr. Hemi-half-kranion, skull).¹⁴

Hemicrania → hemicephalgia — hemi + G.Kaphale, head + algos, pain.